Buy for the bottleneck you can demonstrate

1 / Scope the decision. Original ALHQ buying aid; not a CMS instrument or a deployment approval.

Organization / evaluation date

Revenue cycle / IT / clinical lead

Service / benefit / urgency group

Current bottleneck and evidence reference

Candidate capability; native EHR alternative

Compliance owner: applicable payer / program / rule / date

Distinguish API readiness, hospital reporting and proposals

Approved test environment, synthetic case set and permissions

Excluded uses; no patient identifiers in this workbook

Evaluation window / accountable decision owner / prespecified acceptance criteria

Use controlled, authorized tests and matched synthetic cases. Never submit the same live authorization twice for a comparison. This workbook contains no legal determination, automatic calculation or signed approval.

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Prove coverage and the transaction

2 / Records 1 and 2. Repeat for each payer product and service; a payer logo is not a tested connection.

Payer product / benefit / service / connection version

Tested, planned or unsupported; dated evidence reference

All eligible episodes in the fixed service window

Supported eligible episodes / all eligible episodes; exclusions

Unique synthetic episode ID; requirements and documentation

Submission / acknowledgement / response; evidence references

Missing-information response / correction / subsequent status

Failed attempts, retries and manual fallback; named owner

- [ ] End-to-end evidence retained
- [ ] Partial / unresolved: state gaps
- [ ] Failed / unsupported connection

Distinguish successful transmission from a payer decision. A failed automated attempt followed by manual completion stays visible. Keep request/response logs in your approved repository, not patient details here.

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Test the exception and trace the evidence

3 / Record 3. Proposed synthetic outpatient imaging exercise; no observed vendor result is supplied.

Scenario: the payer requests more information after submission. In an approved test environment, compare the current process and candidate using equivalent synthetic inputs. Define expected behavior before running either workflow.

AI assertion / authorized synthetic source passage / reference

Required information omitted; unsupported statement; correction

Request / missing-information response / correction log IDs

Final or unresolved status; response scope and expiration

Predefined checks: unavailable connection / ambiguous response

Clinical verification / urgency escalation / fallback owner

Reviewer / date / all reviewed packets / errors observed

Disagreement / severity / adjudicator / retest requirement

Human review precedes submission during evaluation. Missing evidence is not permission to invent a chart fact. Count all reviewed packets; a small test cannot establish rare-event safety or universal payer support.

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Measure the work and the cash separately

4 / Records 4 and 5. Repeat per comparable payer, service and urgency group. No automatic totals.

Current process: active person-minutes / eligible episodes

Candidate: active person-minutes / eligible episodes

Review, corrections, retries, calls and fallback included

Incomplete episode count / age; observation window and mix

Verified expense reduction / collected contribution; evidence

Incremental cash cost: licenses, interfaces, implementation

Training, support and monitoring costs; recurring vs one-time

Existing salaried effort / released capacity as opportunity cost

Finance owner / period / net cash calculation and assumptions; subtract each cost once

Sum staff work without double-counting one person's overlapping time. Keep unresolved episodes visible. Released capacity is not booked savings; faster authorization is not collected revenue. State whether benefits are measured or projected.

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Make a bounded procurement decision

5 / Save the decision, its owner and the evidence still needed. Fields do not confer approval.

- ○ Support only the defined scope
- ○ Continue controlled evaluation
- ○ Hold purchase / remediate gaps

Exact scope / exclusions / rationale; which of the five records support this choice?

Missing evidence / acceptance criteria / retest deadline

Exit clause / stop triggers / configuration-change triggers

Revenue cycle / IT / clinical / finance / compliance owners

Approver / actual decision date / approved evidence reference

Next action owner / due date / monitoring recheck

Full-cost case and separate justification for any added AI layer

Save a completed copy, close and reopen it to verify values. Acrobat Reader or Preview is recommended; browser viewers vary. Keep long logs in approved storage. No totals are calculated and no signature is captured. Share only through approved channels.

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